

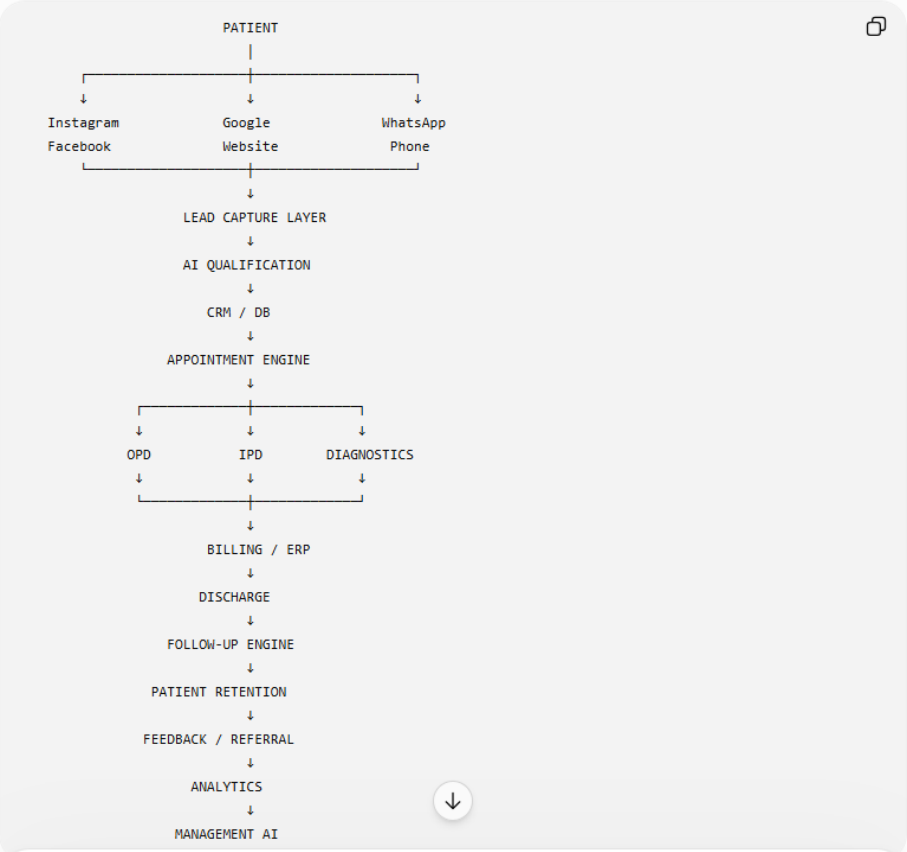
HOSPITAL AI AUTOMATION MASTER BLUEPRINT — V3

A → Z: BUILD • TEST • DEMO • AUDIT • SELL • DELIVER • MAINTAIN

The Sunday Club • @imthatyogii • August 2026

The Entire Architecture

This is the diagram I would use for your Instagram content:



About The Sunday Club

- This blueprint is being shared through The Sunday Club as a practical implementation manual for building and selling AI automation services.
- Instagram: @imthatyogii
- Official Sunday Club community link: [INSERT THE CURRENT OFFICIAL SUNDAY CLUB WHATSAPP/COMMUNITY LINK IN YOUR DM AUTOMATION BEFORE DELIVERY]. Do not hard-code an old link into the PDF.
- This document is intentionally focused on the BUILD + TEST + DEMO + AUDIT + SELL + DELIVER system. It is not the 30 Days / 30 Industries content plan.

How to Use This Blueprint

- Do not try to build the entire hospital at once. Build the Demo Stack first, prove three workflows, then sell a paid pilot.
- Every workflow in this document is described as Trigger → Data → AI/Rules → Action → Human fallback → Log → KPI.
- Use synthetic data for public demonstrations. Production deployment requires the hospital's own approved systems, credentials, policies, security review and vendor/API support.
- Recommended first build: Lead → AI Qualification → Appointment → Confirmation → Reminder → No-Show Recovery.
- Recommended second build: Discharge → Approved Follow-Up → Feedback → Service-Recovery Task.
- Recommended third build: Diagnostic Pending → Staff Escalation → Patient Administrative Notification.

A. Product Definition

- Product name: Hospital Revenue & Patient Lifecycle Automation Layer.
- What it does: connects lead capture, appointment coordination, reminders, no-show recovery, diagnostics follow-up, discharge administration, feedback, repeat-visit workflows and management reporting around the hospital's existing HMS/EMR/CRM.
- What it does NOT do: replace the HMS/EMR, diagnose, prescribe, change treatment, interpret clinical results autonomously or make clinical triage decisions.
- Commercial positioning: ■50,000 starter implementation; ■1,00,000 broader implementation; recurring maintenance/optimization fee. Price is scope-based, not a guaranteed ROI claim.

B. Reference Architecture

- Channel Layer: website/form, Google/Meta lead sources, WhatsApp, phone/IVR and referral sources.
- Orchestration: Make.com or n8n.
- AI Layer: Claude/Anthropic API or another approved LLM.
- Data Layer: hospital HMS/EMR/CRM first; demo database second.
- Communication: approved WhatsApp Business/API provider and email/internal alerts.
- Integration: REST APIs, webhooks, database connectors and optionally MCP for AI tool access.
- Analytics: operational dashboard and exception queue.
- Human Layer: reception, coordinator, department owner, administrator and clinical approver where applicable.

C. Complete Module Map

- 1 Lead Capture
- 2 Lead Qualification
- 3 Department Routing
- 4 Appointment Availability
- 5 Appointment Booking
- 6 Confirmation

- 7 Reminder
- 8 Cancellation / Reschedule
- 9 No-Show Recovery
- 10 OPD Journey Tracking
- 11 Diagnostic Follow-Up
- 12 Billing/Payment Status Coordination
- 13 IPD/Admission Administration
- 14 Discharge Administration
- 15 Post-Discharge Follow-Up
- 16 Feedback
- 17 Service Recovery
- 18 Repeat Visit / Preventive Reminder
- 19 Referral Engine
- 20 Lead SLA Escalation
- 21 Admin Daily Report
- 22 Department Performance
- 23 Exception Queue
- 24 AI Operations Assistant
- 25 Audit / Logging
- 26 Maintenance / Monitoring

D. Build the Demo Environment Before Connecting a Hospital

- Create a fictional facility: 'DemoCare Multispeciality Hospital'.
- Create departments: Cardiology, Dermatology, Orthopedics, General Medicine.
- Create three demo doctors per department only if your calendar demo needs them.
- Create a demo calendar with 5–10 future slots.
- Create a demo database table/Sheet with Patient, Lead, Appointment, FollowUp, CommunicationLog and Exception tables.
- Create a demo form with: full_name, phone, department, preferred_date, enquiry_text, source.
- Create test patient IDs: DEMO-001, DEMO-002, DEMO-003.
- Use a test communication channel or provider sandbox where possible. Never put real patient data into a public demo.

E. Data Model — Exact Fields

- PATIENT: patient_id, full_name, phone, email, city, source, consent_status, created_at, updated_at.
- LEAD: lead_id, patient_id, source, campaign, enquiry_text, department, priority, owner, status, first_response_at, appointment_id, created_at.
- APPOINTMENT: appointment_id, patient_id, department, doctor_id, slot_start, slot_end, status, reminder_24h, reminder_3h, attendance_status, created_at.
- FOLLOW_UP: followup_id, patient_id, category, approved_date, approved_window, owner, status, attempt_count, next_action_at.
- DIAGNOSTIC_TASK: diagnostic_id, patient_id, encounter_id, test_category, ordered_at, completed_at, report_ready_at, review_status, notification_status, owner.
- COMMUNICATION_LOG: message_id, patient_id, channel, template_name, workflow_name, sent_at, delivery_status, response_status, correlation_id.
- EXCEPTION: exception_id, workflow_name, record_id, error_type, severity, owner, due_at, status, resolution_note, created_at.

- AUDIT_LOG: event_id, workflow_name, workflow_version, actor_type, action, record_id, timestamp, correlation_id.

F. Naming Convention

- Workflow names: HOSPITAL | 01 Lead Intake, HOSPITAL | 02 Appointment, HOSPITAL | 03 No-Show, etc.
- Environment names: DEMO, STAGING, PRODUCTION.
- Correlation ID: HOSP-YYYYMMDD-XXXXXX.
- Prompt versions: PROMPT_LEAD_01, PROMPT_MESSAGE_01, PROMPT_SUMMARY_01.
- Message templates: APPT_CONFIRM_01, APPT_REMINDER_01, APPT_RESCHEDULE_01, NOSHOW_RECOVERY_01, FEEDBACK_01.

G. AI Guardrails — Put This Before Every AI Prompt

- System role: administrative hospital automation assistant.
- Allowed tasks: classification, extraction, summarization, administrative message drafting, routing suggestions and operational reporting.
- Forbidden tasks: diagnosis, treatment recommendations, medication changes, clinical interpretation, autonomous triage or inventing facts.
- Data rule: use only supplied data; never guess missing facts.
- Escalation rule: ambiguous, clinically sensitive or high-impact requests return NEEDS_HUMAN_REVIEW.
- Output rule: strict JSON when consumed by automation.
- Privacy rule: minimum necessary data only; do not echo unnecessary patient information.

H. MASTER SYSTEM PROMPT — COPY/PASTE

```
You are an administrative automation assistant for a hospital or clinic.

MISSION
Process approved administrative workflows accurately and safely.

YOU MAY
- classify patient enquiries into an administrative department/category
- extract structured fields from supplied text
- draft approved administrative messages
- summarize operational metrics
- create or recommend administrative follow-up tasks
- select an approved tool when the workflow explicitly allows tool use

YOU MUST NOT
- diagnose a patient
- recommend treatment
- change medication
- interpret clinical results
- make clinical triage decisions
- invent appointment slots
- invent patient facts
- expose information that is not necessary for the requested action

DATA RULES
1. Use only facts present in the input or returned by approved tools.
2. If information is missing, return NEEDS_HUMAN_REVIEW.
3. If a request is clinical, return NEEDS_HUMAN_REVIEW.
4. Never create an appointment unless the scheduling system confirms the slot.
5. Log the workflow name and correlation_id when provided.

OUTPUT
Return only the JSON schema requested by the workflow. No markdown. No extra commentary.
```

I. Prompt 1 — Lead Qualification

```
TASK
Classify this incoming hospital enquiry.
```

```

INPUT
name: {{full_name}}
phone: {{phone}}
department_selected: {{department}}
enquiry_text: {{enquiry_text}}
source: {{source}}

RETURN JSON
{
  "department": "",
  "enquiry_type": "",
  "patient_status": "new|existing|unknown",
  "priority_for_human_review": "normal|review",
  "missing_information": [],
  "reason": ""
}

RULES
- Do not diagnose.
- Do not recommend treatment.
- If the enquiry contains a clinical emergency or clinical question, set priority_for_human_review=review.
- Do not invent a department. If uncertain, use "NEEDS_HUMAN_REVIEW".

```

J. Prompt 2 — Appointment Message

```

TASK
Draft a short administrative appointment confirmation.

FACTS
patient_name: {{patient_name}}
department: {{department}}
doctor_name: {{doctor_name}}
confirmed_slot: {{confirmed_slot}}
hospital_name: {{hospital_name}}
hospital_contact: {{hospital_contact}}

RULES
- Use only the supplied facts.
- Do not add clinical advice.
- Do not change the date/time.
- Keep it concise.
- Return JSON:
{
  "status": "READY|NEEDS_HUMAN_REVIEW",
  "message": ""
}

```

K. Prompt 3 — No-Show Recovery

```

TASK
Draft an administrative rescheduling message.

FACTS
patient_name: {{patient_name}}
appointment_date: {{appointment_date}}
hospital_name: {{hospital_name}}
reschedule_link_or_options: {{reschedule_link_or_options}}

RULES
- Do not mention medical reasons.
- Do not pressure the patient.
- Do not invent availability.
- Ask whether they would like to reschedule.
- Return JSON with status and message.

```

L. Prompt 4 — Follow-Up Task

```

TASK
Create an administrative follow-up task from an approved hospital instruction.

FACTS
patient_id: {{patient_id}}

```

```
approved_followup_date: {{approved_followup_date}}
category: {{category}}
owner: {{owner}}
```

```
RETURN
{
  "task_title": "",
  "due_date": "",
  "owner": "",
  "status": "READY|NEEDS_HUMAN_REVIEW"
}
```

Do not infer a medical schedule.

M. Prompt 5 — Daily Management Summary

TASK
Summarize only the supplied operational metrics.

INPUT
{{metrics_json}}

```
RETURN
{
  "headline": "",
  "wins": [],
  "exceptions": [],
  "overdue_items": [],
  "questions_for_management": []
}
```

Do not infer clinical conclusions. Do not invent metrics.

N. Prompt 6 — Error / Exception Classifier

TASK
Classify an automation failure.

INPUT
workflow: {{workflow}}
failed_step: {{failed_step}}
error: {{error}}
record_id: {{record_id}}

```
RETURN
{
  "severity": "low|medium|high",
  "safe_to_retry": true,
  "human_review_required": false,
  "reason": "",
  "recommended_action": ""
}
```

Never recommend an automatic retry for an action that could duplicate a patient message, appointment, payment or other high-impact action unless idempotency is confirmed.

O. Make.com — Click-by-Click Demo Build

- 1. Create a Make account and open a new Scenario.
- 2. Click the large + button → search 'Webhooks' → choose 'Custom webhook'.
- 3. Click Add → name it HOSPITAL_DEMO_LEAD_IN → Save.
- 4. Copy the webhook URL.
- 5. Open your demo form tool and configure its submission webhook to POST to that URL. If the form tool has a native Make trigger, use that instead.
- 6. In Make, click Run once.
- 7. Submit the demo form with DEMO-001. Confirm the webhook receives the sample bundle.

- 8. Click the output bubble and inspect every field. Do not continue until the sample data is correct.
- 9. Click + after the webhook → add JSON/Tools normalization if needed.
- 10. Add the AI step. If using Make AI Agent (new), choose the approved provider/model and paste the Master System Prompt + Lead Qualification Prompt. Map the form fields into the input.
- 11. Run once and inspect the AI output. Confirm department, patient_status and missing_information are present.
- 12. Add Router. Create routes for Cardiology, Dermatology, Orthopedics, General Medicine and HUMAN_REVIEW.
- 13. On each route, add your demo database/CRM 'Create/Update record' module.
- 14. Add the appointment lookup step. If your calendar/HMS has a native connector, use it. Otherwise use HTTP/API only after obtaining the vendor's documented API.
- 15. Add a filter: only continue if the requested slot exists in returned data.
- 16. Create/update the Appointment record.
- 17. Add the approved message-generation prompt or deterministic template.
- 18. Add the WhatsApp/API send step using your approved provider. For public demo, use a sandbox/test recipient.
- 19. Add the Communication Log record.
- 20. Add the reminder mechanism. Use a scheduler/queue approach rather than sleeping a workflow for long periods when the platform's scheduling model is more appropriate.
- 21. Turn on error handling. Route failures to EXCEPTION.
- 22. Save and name the scenario HOSPITAL | 01 Lead Intake.
- 23. Activate only after the full demo test passes.

P. Make.com — Webhook Test Payload

```
POST JSON
{
  "full_name": "Rahul Sharma",
  "phone": "9999999999",
  "department": "Dermatology",
  "preferred_date": "2026-09-05",
  "enquiry_text": "I want to book a consultation.",
  "source": "Instagram",
  "correlation_id": "HOSP-20260905-DEM0001"
}
```

Expected:

Webhook receives data → AI returns department=Dermatology → lead record created → appointment availability checked → confirmed slot only if returned by scheduler → message generated → communication logged.

Q. Make.com — No-Show Workflow

- 1. Create new Scenario: HOSPITAL | 03 No-Show Recovery.
- 2. Trigger from your appointment system's status update/webhook or a scheduled database search for appointments whose attendance_status=no_show.
- 3. Lookup appointment by appointment_id.
- 4. Verify current status is still no_show. Stop if status changed.
- 5. Check whether a recovery message was already sent. Stop if yes.
- 6. Generate the approved administrative message using Prompt 3 or use a fixed approved template.
- 7. Send via approved WhatsApp/API route.
- 8. Log message_id, template_name, timestamp and correlation_id.
- 9. Create follow-up task if there is no response within the defined window.
- 10. Escalate to staff after the maximum retry count.

- 11. KPI: recovered appointments / eligible no-shows.

R. Make.com — Discharge Follow-Up Workflow

- 1. Trigger from approved discharge event.
- 2. Retrieve only operational fields required for communication.
- 3. Confirm the hospital-approved follow-up date/window.
- 4. Create follow-up task.
- 5. Send approved administrative discharge/follow-up message.
- 6. Schedule reminder.
- 7. Record communication.
- 8. After the approved interval, request service feedback.
- 9. Route negative feedback to an internal service-recovery queue.
- 10. Never let the AI invent clinical instructions.

S. n8n — Click-by-Click Demo Build

- 1. Open n8n and create a new Workflow.
- 2. Add Webhook node. Set HTTP Method=POST and Path=hospital-demo-lead.
- 3. Save and use the Test URL while building. Send the same sample JSON from Section P.
- 4. Execute the workflow once and inspect the Webhook output.
- 5. Add Edit Fields/Set node to normalize names, phone, department, date and enquiry_text.
- 6. Add the appropriate AI/LLM node or AI Agent node. Create the credential using the approved provider/API key. Do not paste secrets into the prompt.
- 7. Paste the Master System Prompt and Lead Qualification Prompt. Map fields from the Edit Fields node.
- 8. Require structured output. Validate the JSON before routing.
- 9. Add Switch node. Route by department.
- 10. Add your database/CRM node or HTTP Request node for the hospital's API.
- 11. Add HTTP Request for scheduling only when an approved API exists. Configure authentication through n8n credentials.
- 12. Add IF node: available slot exists? If false → human review path.
- 13. Create/update appointment record.
- 14. Add approved communication provider node or HTTP Request.
- 15. Add logging node.
- 16. Add error workflow / error handling and an Exception record.
- 17. Save workflow as HOSPITAL | 01 Lead Intake.
- 18. Activate only after testing all cases in Section T.

T. n8n — Suggested Node Map

- Webhook → Edit Fields → AI/LLM → Structured Output Validator → Switch → CRM/DB → HTTP Request (availability) → IF → Appointment DB → Message Template/AI → HTTP Request/WhatsApp → Communication Log.
- For no-show: Trigger → Lookup Appointment → IF current status=no_show → IF recovery already sent? → Message → Log → Wait/Schedule → Check response → Follow-up Task → Human Escalation.
- For discharge: Trigger → Get approved follow-up data → Create Task → Message → Log → Schedule reminder → Feedback → Exception/Service Recovery.

U. Claude Setup — Two Practical Routes

- Route 1 — Prompt testing: use Claude/Anthropic Console/Workbench to test prompts with synthetic cases before putting them into Make/n8n.
- Route 2 — Programmatic: create an Anthropic API credential in the Anthropic Console and connect it to your automation platform. Keep the API key in the platform's credential store.
- Do not use Claude Code as the hospital workflow engine. Claude Code is a developer tool; the production workflow should run in Make/n8n or the hospital's approved application infrastructure.
- If using Claude as an MCP client, expose narrow tools with explicit permissions. Keep write actions behind approval where appropriate.

V. MCP — Build Three Tools First

- Tool 1: `get_appointment_slots(input: department, date, duration) → output: available_slots[]`
- Tool 2: `create_followup_task(input: patient_id, category, due_date, owner) → output: task_id, status`
- Tool 3: `get_daily_operations_summary(input: date, department_optional) → output: metrics`
- Implement each tool with strict schemas, authentication, authorization, audit logs and a clear error response.
- Connect the tools to Claude only after they work independently.
- Do not give an AI agent unrestricted access to patient tables.

W. WhatsApp — Production Setup Logic

- Choose either Meta's WhatsApp Business Platform/Cloud API directly or an approved Business Solution Provider that fits the hospital's requirements.
- Create/verify the business assets required by the selected provider.
- Create/approve message templates for confirmation, reminder, reschedule, no-show recovery and feedback.
- Configure the provider's webhook for inbound events if two-way automation is required.
- Store provider credentials only in the automation platform's secure credential store.
- Use the provider's documented send-message endpoint/module and map the recipient, template name, language and approved variables.
- Test with a dedicated test number before production.
- Do not send real patient data until the hospital has approved the communication flow and applicable consent/privacy requirements.

X. HMS/EMR Integration Discovery — Do Not Guess

- Ask the hospital for the HMS/EMR product name and vendor contact.
- Ask for API documentation, webhook documentation, sandbox/test credentials and rate limits.
- Identify whether the required data can be read, written or only exported.
- Create an Integration Matrix: system, endpoint/event, read/write, fields, authentication, owner, test status.
- Never assume an HMS has an API because it is a modern product. Verify.
- If no API exists, consider a supported export/import or a human-operated handoff rather than an unsafe workaround.
- Any production integration must be approved by the hospital/vendor.

Y. Integration Matrix Template

- System: HMS/EMR | Event: Appointment Created | Direction: HMS→Automation | Data: appointment_id, patient_id, slot | Auth: vendor credential | Owner: HMS vendor | Test: pending.
- System: CRM | Event: Lead Created | Direction: Automation→CRM | Data: lead_id, source, department | Auth: API | Owner: hospital admin | Test: pending.
- System: WhatsApp | Event: Send Message | Direction: Automation→Provider | Data: phone, template, variables | Auth: provider credential | Test: pending.

- System: Calendar | Event: Slot Lookup | Direction: Automation→Calendar | Data: doctor, date | Auth: OAuth/API | Test: pending.

Z. Error Handling & Idempotency

- Every write action needs a duplicate-prevention key where possible.
- Lead key: phone + normalized date/source or a hospital patient/lead ID.
- Appointment key: appointment_id.
- Message key: appointment_id + template_name + reminder_stage.
- Before sending a message, check the communication log for an existing successful send.
- Before creating an appointment, verify the appointment ID/slot status from the source system.
- Every failure creates an Exception record with severity, owner and next action.
- Never build a workflow that silently fails.

AA. Dashboard — Minimum Version

- Daily: new leads, qualified leads, booked appointments, confirmations, no-shows, recovered no-shows, pending follow-ups, exceptions.
- Weekly: lead-to-appointment conversion, no-show rate, recovery rate, first-response time, department distribution.
- Management view: trend lines, top exceptions, overdue tasks, lead source performance and workflow health.
- Do not show sensitive clinical data on a general marketing dashboard. Use minimum necessary operational data.

AB. Audit — The Paid Discovery Product

- Step 1: interview owner/admin/reception/marketing/IT.
- Step 2: map current patient journey.
- Step 3: list systems and manual steps.
- Step 4: measure leakage.
- Step 5: score each automation opportunity on revenue impact, frequency, technical feasibility and risk.
- Step 6: choose one pilot workflow.
- Step 7: produce a 3-page audit: current state, leakage, proposed automation, KPI and scope.
- Use the audit to sell the implementation rather than selling 'AI' generically.

AC. Hospital Automation Audit Scorecard

- Lead response time: 1–5.
- Lead capture consistency: 1–5.
- Appointment conversion visibility: 1–5.
- No-show recovery: 1–5.
- Follow-up tracking: 1–5.
- Diagnostic coordination: 1–5.
- Discharge coordination: 1–5.
- Feedback/service recovery: 1–5.
- Reporting automation: 1–5.
- Integration readiness/API availability: 1–5.
- Security/governance readiness: 1–5.
- Highest score does not automatically mean highest priority; prioritize by impact × frequency × feasibility × risk.

AD. Lead Generation — Build the Prospect List

- Create a spreadsheet/CRM with: facility, city, website, public contact, decision-maker role, source, likely problem, outreach status, last contact, next action.
- Prioritize hospitals and clinics with multiple departments, active online acquisition and visible appointment/enquiry processes.
- Use public business information responsibly and follow platform terms and applicable outreach rules.
- Start with 50 prospects in one city rather than 1,000 prospects with no personalization.

AE. Cold Outreach — Copy/Paste

```
DM:
Hi [Name], I was looking at [Hospital] and noticed you are actively handling patient enquiries online. I
build automation systems that connect lead capture, appointment booking, reminders and follow-up around an
existing HMS. I have a working demo. Would you be open to a 10-minute walkthrough?

Email subject:
A simple patient-follow-up automation idea for [Hospital]

Body:
Hi [Name],
I build workflow automation for hospitals/clinics around their existing systems. The goal is to reduce
manual lead follow-up, appointment reminders and missed-appointment recovery.
I have a demo workflow I can show in 10 minutes. If it looks useful, we can identify one process for a
small pilot.
— Yogii

Follow-up:
Just following up on this. The important part is that this does not need to replace your HMS; it connects
the gaps around it. Happy to show the live workflow.
```

AF. Discovery Call Script

- Opening: 'Before I show you anything, I want to understand where the patient journey currently breaks.'
- Ask: monthly enquiries, sources, response time, bookings, cancellations, no-shows, follow-up process, diagnostics, discharge, reporting.
- Ask: 'If you could remove one manual process tomorrow, what would it be?'
- Reflect back the problem in their language.
- Show only the relevant demo workflow.
- Ask: 'If this worked exactly as shown, where would this save your team the most time or recover the most appointments?'
- Recommend a pilot with a defined KPI.
- Then present price and implementation scope.

AG. Offer & Pricing

- STARTER ■50,000: discovery + one core workflow + lead capture/appointment + reminder + basic no-show recovery + logging/report + training/handover.
- GROWTH ■1,00,000: multiple connected workflows + department routing + follow-up + diagnostic/discharge coordination where feasible + exception queue + management dashboard + training/handover.
- MAINTENANCE: monthly monitoring, error checks, template changes, credential/API checks, small changes and monthly optimization.
- Use a custom quote when HMS/API complexity, multiple facilities, large departments or custom software is involved.

AH. Proposal — Exact Sections

- 1 Cover
- 2 Current problem
- 3 Current-state workflow
- 4 Leakage identified

- 5 Proposed workflow
- 6 Architecture
- 7 Scope
- 8 Integration assumptions
- 9 Deliverables
- 10 Testing
- 11 Timeline
- 12 Client responsibilities
- 13 Security/data responsibilities
- 14 Investment
- 15 Maintenance
- 16 Exclusions
- 17 Acceptance/sign-off

AI. Closing Framework

- Do not close on 'AI'. Close on the defined problem and pilot outcome.
- Question: 'Would you like us to start with the highest-leakage workflow rather than changing everything at once?'
- Question: 'If we agree on this scope, can we lock the implementation start date?'
- After verbal agreement: send proposal + payment terms + onboarding checklist.
- Do not start production work before the agreed commercial and access requirements are satisfied.
- Never guarantee a fixed revenue increase. Commit to a measurable workflow and reporting framework.

AJ. Objection Handling

- 'We already have an HMS.' → 'Good. This is an automation layer around it, not a replacement.'
- 'Our staff can do this.' → 'Absolutely. The goal is to remove repetitive coordination so staff can focus on exceptions and patient conversations.'
- '■1 lakh is expensive.' → 'The scope is the number of workflows, integrations, testing and handover. We can start with the ■50K pilot around one measurable problem.'
- 'Can you guarantee revenue?' → 'No responsible automation provider should guarantee a number before seeing the baseline. We can define KPIs and measure the result.'
- 'Can you integrate our HMS?' → 'I can confirm after reviewing the vendor's API/webhook capabilities and permissions.'

AK. Delivery Checklist

- Signed scope.
- Payment/advance per contract.
- Hospital owner and technical contact.
- HMS/EMR details.
- API documentation and test credentials.
- Approved WhatsApp provider.
- Approved message templates.
- Test patient data.
- Department/doctor mapping.
- Calendar/scheduling access.

- Exception owner.
- Go-live date.
- Rollback plan.
- Training session.
- Maintenance plan.

AL. Production Go-Live Procedure

- 1 Freeze the tested workflow version.
- 2 Export/back up workflow configuration where supported.
- 3 Replace demo credentials with hospital-approved production credentials.
- 4 Confirm webhook URLs and security controls.
- 5 Test with a controlled internal record.
- 6 Test one real-but-authorized transaction with staff observation if the hospital approves.
- 7 Monitor execution logs.
- 8 Confirm message delivery.
- 9 Confirm CRM/HMS write-back.
- 10 Confirm exception alerts.
- 11 Sign off with hospital owner.
- 12 Monitor closely during the first operating period and review failures.

AM. Maintenance SOP

- Daily/automated: workflow failure alerts.
- Weekly: review failed runs, exceptions, delivery failures and duplicate events.
- Monthly: KPI review, template review, credential/API health check and improvement backlog.
- Quarterly: access review, workflow version review and integration/vendor changes.
- Keep a change log for every production modification.

AN. What You Should Actually Build for the Public Demo

- Demo 1: Lead form → Make/n8n → Claude classification → database → appointment availability → confirmation.
- Demo 2: change appointment status to no_show → recovery message → follow-up task.
- Demo 3: change discharge status → approved follow-up → reminder → feedback → service-recovery ticket.
- Do not build 26 modules before selling. Build the three demonstrations, publish them, then use the audit to customize the rest for a real hospital.

AO. Public Demo Video Shot List

- Shot 1: you on camera — 'I built a hospital automation system that can be sold for ■50K–■1L.'
- Shot 2: submit a demo patient form.
- Shot 3: show the workflow trigger.
- Shot 4: show AI classification.
- Shot 5: show database update.
- Shot 6: show appointment slot returned by the demo scheduler.
- Shot 7: show confirmation message.
- Shot 8: switch status to no-show.

- Shot 9: show recovery workflow.
- Shot 10: zoom out to the complete architecture.
- Shot 11: CTA — 'Comment HOSPITAL and I'll send the complete blueprint.'

AP. DM Automation Delivery Flow

- Trigger: comment contains HOSPITAL.
- DM 1: acknowledge and deliver the blueprint link.
- DM 2: invite the person to The Sunday Club using the current official community link.
- DM 3, optional: ask whether they want the Make.com or n8n implementation path.
- Do not make the community invite a condition for receiving the promised blueprint.
- Keep the delivery message short; the blueprint is the detailed asset.

AQ. Final Build Checklist — Zero to Sellable System

- ☐ Create demo hospital.
- ☐ Create demo form.
- ☐ Create demo database.
- ☐ Create demo calendar.
- ☐ Create Make.com workflow OR n8n workflow.
- ☐ Connect AI provider.
- ☐ Paste and test master prompt.
- ☐ Test lead classifier.
- ☐ Test appointment lookup.
- ☐ Test confirmation.
- ☐ Test no-show recovery.
- ☐ Test discharge follow-up.
- ☐ Add exception queue.
- ☐ Add communication log.
- ☐ Create dashboard.
- ☐ Record live demo.
- ☐ Create hospital audit.
- ☐ Create proposal.
- ☐ Build 50-prospect list.
- ☐ Send personalized outreach.
- ☐ Run discovery calls.
- ☐ Sell pilot.
- ☐ Onboard client.
- ☐ Connect approved production systems.
- ☐ Test with hospital.
- ☐ Go live.
- ☐ Monitor.
- ☐ Sell maintenance/optimization.

AR. Important Healthcare & Data Boundary

- This manual is an implementation framework, not legal, medical, cybersecurity or accreditation advice.
- For production, the hospital's authorized privacy/security/IT/clinical stakeholders must approve the workflow and data handling.
- Use minimum necessary data, role-based access, secure credentials, audit logs and approved communication channels.
- ABDM describes an integrated digital-health ecosystem with building blocks including ABHA, HFR, HPR, PHR and UHI, and emphasizes consent-based secure exchange. Treat ABDM work as a separate integration scope where applicable.
- NABH digital-health standards and the hospital's own policies should be checked for the applicable implementation context.

AS. Official Documentation to Keep Open While Building

- Make Webhooks: <https://help.make.com/webhooks>
- Make Webhook-triggered AI Agent: <https://help.make.com/webhook-triggered-ai-agent>
- Make Scenarios for AI Agents: <https://help.make.com/scenarios-for-ai-agents>
- Make Scenario Inputs & Outputs: <https://help.make.com/scenario-inputs-and-outputs>
- Anthropic / Claude documentation: <https://docs.anthropic.com/>
- Anthropic Claude Code setup/reference: <https://docs.anthropic.com/en/docs/claude-code/getting-started>
- n8n documentation: <https://docs.n8n.io/>
- Model Context Protocol: <https://modelcontextprotocol.io/>
- Meta WhatsApp Business Platform: <https://developers.facebook.com/docs/whatsapp/>
- ABDM: <https://abdm.gov.in/>
- NABH: <https://nabh.co/digital-health-standards/>

Implementation Reference Table

Layer	Demo	Production	Owner
Lead capture	Tally/website form	Hospital website/approved source	Marketing/Front desk
Workflow	Make.com or n8n	Hospital-approved Make/n8n deployment	Automation owner
AI	Claude/Anthropic API	Approved model/API account	Automation + IT
Data	Demo DB/Sheet	HMS/EMR/CRM	IT/Admin
Scheduling	Demo Calendar	HMS/booking API	Admin/IT
Messaging	Sandbox/test route	Approved WhatsApp Business/API	Admin/IT
Exceptions	Demo table	Hospital ticket/CRM	Department owner
Analytics	Simple dashboard	Approved BI/dashboard	Management

Blueprint Completion Standard

A person following this manual should be able to create a synthetic-data demo, execute the three core workflows, test failure paths, present the live demo, conduct a hospital automation audit, scope a ■50K–■1L implementation, create a proposal, close a pilot and hand over a documented system.

Exact UI labels, API versions, model names, WhatsApp provider requirements and hospital-vendor capabilities can change. Verify the live official documentation and the client's systems immediately before implementation.